

100-0-3
carcinoma, papillary, thyroid 8260/3
Site: thyroid, NOS C73.9

for
4/26/11

Surgical Pathology Consultation Report

Patient Name:		Accession #:
MRN:		Collected:
DOB:		Received:
Gender: M	Service:	Reported:
HCN:	Visit #:	
Ordering MD:	Location:	
Copy To:	Facility:	

Specimen(s) Received

1. Lymph-Node: It paratracheal nodes
2. Trachea: Tracheal resection
3. Thyroid: Total thyroidectomy stitch marks rt. superior

Diagnosis

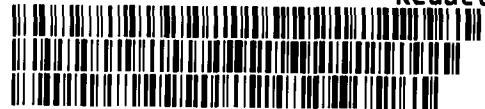
1. Metastatic papillary thyroid carcinoma: Lymph node (left paratracheal node) excisional biopsy
2. Infiltrative papillary thyroid carcinoma: Trachea excisional biopsy
3. Multifocal papillary carcinoma, dominant angioinvasive and widely invasive classical variant with focal dedifferentiation (5%) and tall cell change (10%), involving surgical margins, 6.6 cm, left; and medullary carcinoma, 1.3 cm, right: Thyroid total thyroidectomy specimen

Synoptic Data

Procedure:	Total thyroidectomy
Received:	Fresh
Specimen Integrity:	Intact
Specimen Size:	Right lobe:
	4.1 cm
	2.3 cm
	1.5 cm
	Left lobe:
	6.6 cm
	4.2 cm
	3.5 cm
	Isthmus +/- pyramidal lobe:
	1.2 cm
	1.6 cm
	0.2 cm

UUID: 3D5E6A4E-1626-414F-91BC-009A03C57016
TCGA-EM-A220-01A-PR

Redacted



Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewed by:	for 4/25/11	for 4/26/11

Specimen Weight:	63.7 g
Tumor Focality:	Multifocal, bilateral Multifocal, midline (isthmus)
----- DOMINANT TUMOR -----	
Tumor Laterality:	Left lobe
Tumor Size:	Greatest dimension: 6.6cm Additional dimension: 4.2 cm Additional dimension: 3.5 cm
Histologic Type:	Papillary carcinoma, classical (usual) Classical (papillary) architecture Other architecture: with focal solid growth, 5% Classical cytomorphology Tall cell cytomorphology Other: with focal dedifferentiation, 5%
Histologic Grade:	Not applicable
Margins:	Margin(s) involved by carcinoma
Tumor Capsule:	Totally encapsulated
Tumor Capsular Invasion:	Present, extent widely invasive
Lymph-Vascular Invasion:	Present, focal extent (less than 4 vessels)
Perineural Invasion:	Not identified
Extrathyroidal Extension:	Present, extensive
----- SECOND TUMOR -----	
Tumor Laterality:	Right lobe
Tumor Size:	Greatest dimension: 0.1cm
Histologic Type:	Papillary carcinoma, follicular variant Papillary carcinoma, microcarcinoma (occult, small or microscopic) variant Follicular architecture Classical cytomorphology
Histologic Grade:	Not applicable
Margins:	Margins uninvolved by carcinoma
Tumor Capsule:	Partially encapsulated
Tumor Capsular Invasion:	Not identified
Lymph-Vascular Invasion:	Not identified
Perineural Invasion:	Not identified
Extrathyroidal Extension:	Not identified
TNM Descriptors:	m (multiple primary tumors)
Primary Tumor (pT):	pT4a: Tumor of any size extending beyond the thyroid capsule to invade subcutaneous soft tissues, larynx, trachea, esophagus or recurrent laryngeal nerve
Regional Lymph Nodes (pN):	pN1a: Nodal metastases to Level VI (pretracheal, paratracheal and prelaryngeal/Delphian) lymph nodes Number of regional lymph nodes examined: 1 Number of regional lymph nodes involved: 1 Lymph Node, Extranodal Extension Not identified
Distant Metastasis (pM):	Not applicable
Additional Pathologic Findings:	Adenomatoid nodule(s) or Nodular follicular disease Other: Additional papillary microcarcinomas identified including 0.1 cm isthmus. Medullary carcinoma, 1.3 cm right.
Ancillary Studies:	Type: IHC Results: The medullary carcinoma stains positive for calcitonin, CEA, chromogranin, and TTF1. It is negative for thyroglobulin and MIB-1 LI is 2%.

*Pathologic Staging is based on AJCC/UICC TNM, 7th Edition

Procedure:	Total thyroidectomy
Received:	Fresh
Specimen Integrity:	Intact
Specimen Size:	Right lobe: 4.1 cm 2.3 cm 1.5 cm Left lobe: 6.6 cm 4.2 cm 3.5 cm Isthmus +/- pyramidal lobe: 1.2 cm 1.6 cm 0.2 cm
Specimen Weight:	63.7 g
Tumor Focality:	Unifocal
----- DOMINANT TUMOR -----	
Tumor Laterality:	Right lobe
Tumor Size:	Greatest dimension: 1.3cm Additional dimension: 1.3 cm Additional dimension: 1.3 cm
Histologic Type:	Medullary carcinoma
Histologic Grade:	Not applicable
Margins:	Margins uninvolved by carcinoma
Tumor Capsule:	Partially encapsulated
Tumor Capsular Invasion:	Present, extent minimal
Lymph-Vascular Invasion:	Indeterminate
Perineural Invasion:	Not identified
Extrathyroidal Extension:	Not identified
TNM Descriptors:	Not applicable
Primary Tumor (pT):	pT1b: Tumor more than 1 cm but not more than 2 cm in greatest dimension, limited to the thyroid
Regional Lymph Nodes (pN):	pN0: No regional lymph node metastasis Number of regional lymph nodes examined: 1 Number of regional lymph nodes involved: 0
Distant Metastasis (pM):	Not applicable
Additional Pathologic Findings:	Other: Multifocal papillary carcinoma including 6.6 cm dominant nodule in left lobe with focal dedifferentiation
Ancillary Studies:	Type: IHC Results: The medullary carcinoma stains positive for calcitonin, CEA, chromogranin, and TTF1. It is negative for thyroglobulin and MIB LI is 2%.

*Pathologic Staging is based on AJCC/UICC TNM, 7th Edition

Comment

1. The metastatic lesion in the lymph nodes shows clear architectural and nuclear features of papillary carcinoma.
2. The infiltrative tumor in the trachea stains positive for thyroglobulin and negative for calcitonin, chromogranin and CEA, consistent with papillary carcinoma.

3. The dominant 6.6 cm left lobe tumor is an angioinvasive and widely invasive classical variant papillary carcinoma with focal dedifferentiation and tall cell change involving surgical margins. The tall cell change represents approximately 10% of the tumor and the very focal dedifferentiation represents approximately 5% of this tumor that is characterized by solid growth pattern and apoptotic change.

The medullary carcinoma is strongly positive for TTF-1, chromogranin-A, calcitonin and CEA, and is negative for thyroglobulin. MIB-1 LI is 2%. Since the medullary thyroid carcinoma exhibits local infiltrative growth pattern in the right lobe and the left lobe is almost entirely replaced by papillary carcinoma, the assessment of underlying C-cell hyperplasia cannot be made.

Electronically

verified by:

Clinical History

Thyroid carcinoma

Gross Description

1. The specimen labeled with the patient's name and as "LT paratracheal nodes", consists of a piece of fibrofatty tissue that measures 1.3 x 1.0 x 0.5 cm. Within the tissue is a single lymph node that measures 1.0 cm in maximum dimension. Representative sections, lymph node submitted in toto:

1A 1 lymph node, bisected

2. The specimen labeled with the patient's name and as "Tracheal resection", consists of a piece of cartilaginous tissue that measures 2.0 x 1.9 cm with a maximum thickness of 0.3 cm. There is a central defect, open to one edge that measures 1.2 cm in diameter. No gross lesion is identified.

2A representative sections including edge of defect

3. The specimen labeled with the patient's name and as "Total thyroidectomy stitch marks R. superior" consists of a thyroid gland that is received oriented with a suture and weighs 63.7 g. The right lobe measures 4.1 cm SI x 2.3 cm ML x 1.5 cm AP, the left lobe measures 6.6 cm SI x 4.2 cm ML x 3.5 cm AP and the isthmus measures 1.2 cm SI x 1.6 cm ML x 0.2 cm AP. The external surfaces have fibrous adhesions and are painted with silver nitrate. No parathyroid glands or lymph nodes are identified grossly. The right and left lobes each contains one delineated nodule that measure 1.3 cm SI x 1.3 cm ML x 1.3 cm AP and 6.6 cm SI x 4.2 cm ML x 3.5 cm AP, respectively. The remainder of the thyroid tissue is red-brown. Sections of both nodules and normal tissue are stored frozen. Representative sections of the remainder of the specimen are submitted as follows:

3A-G right lobe, in toto, superior to inferior

3H-I isthmus, in toto

3J-R left lobe, superior to inferior
